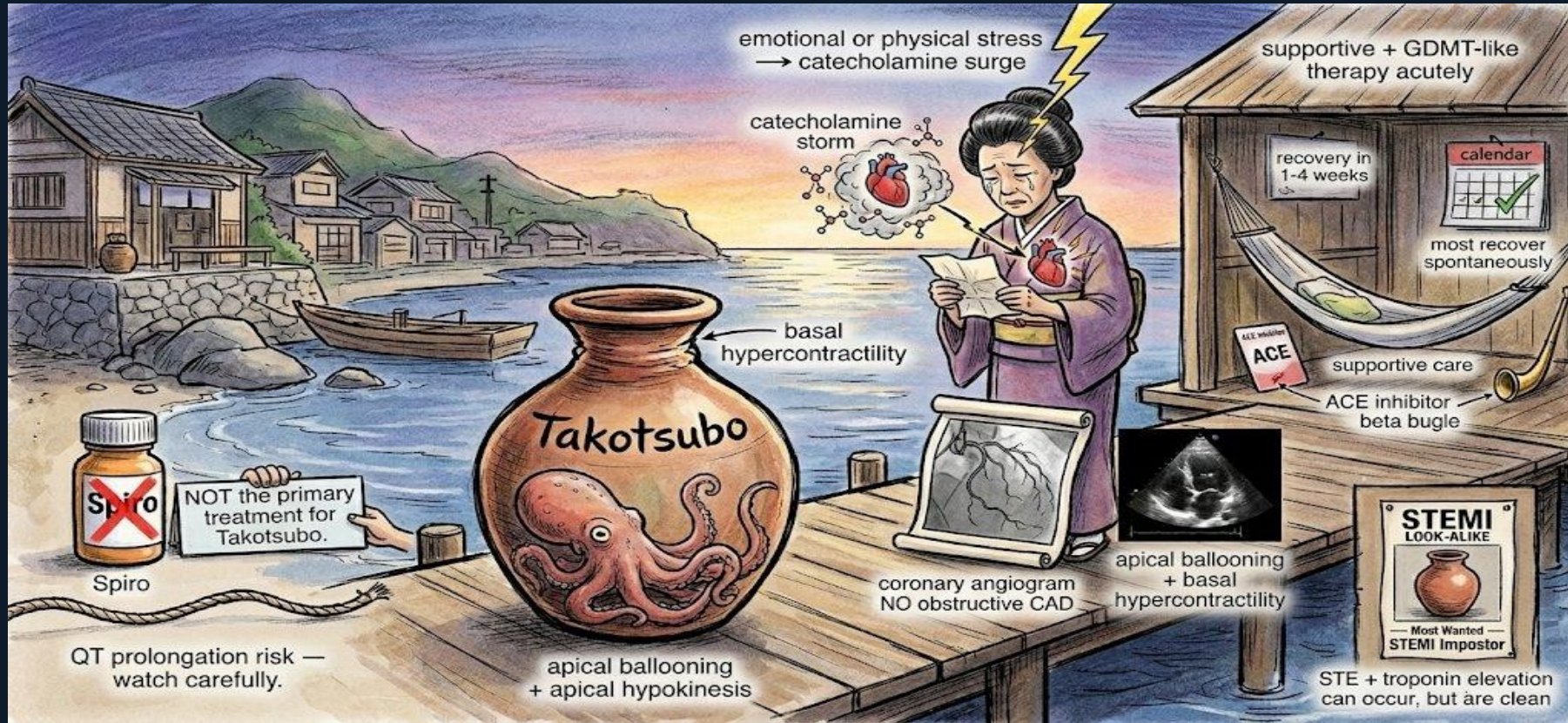
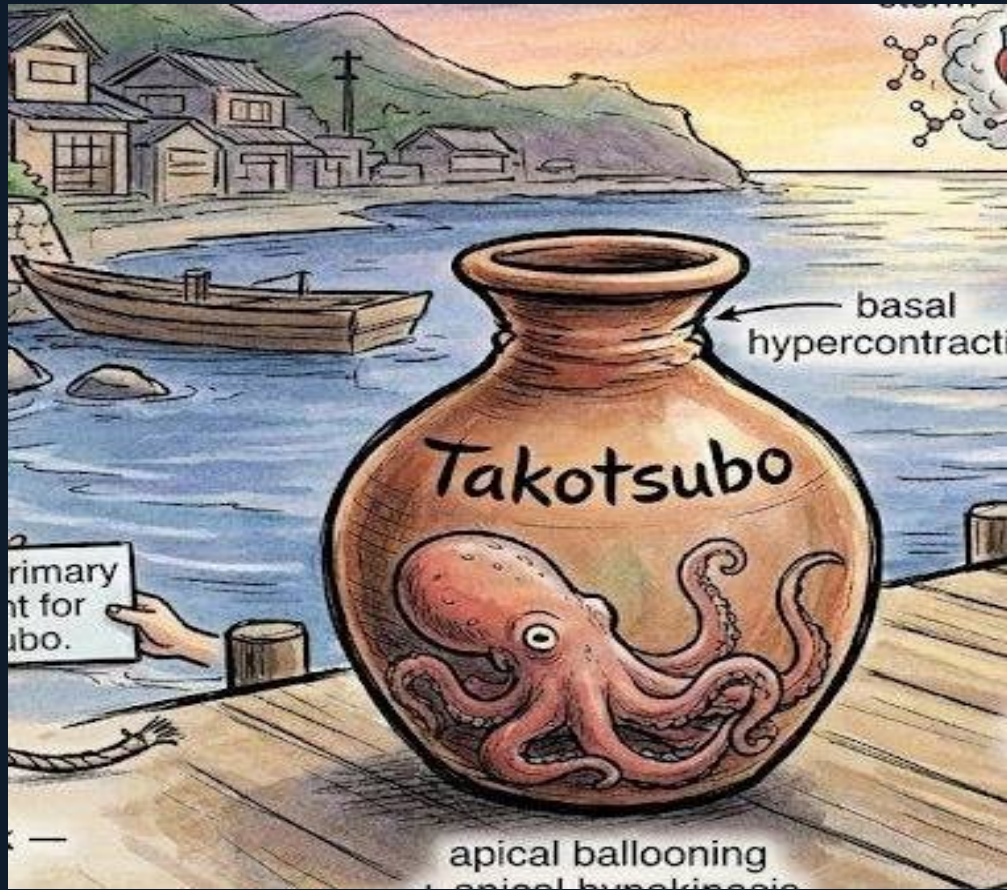


Scene F — Takotsubo (Stress-Induced CM)



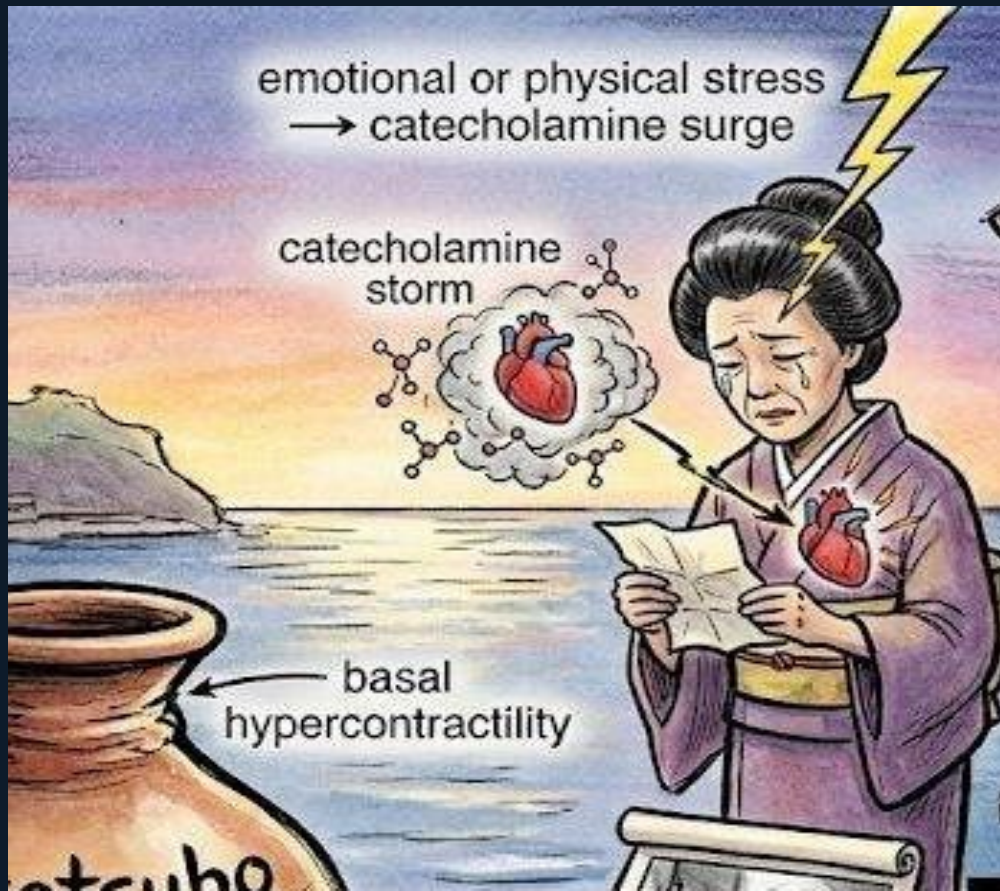
Japanese Fishing Village · 3 board questions



Scene F — Takotsubo | Japanese Fishing Village

The Takotsubo Pot

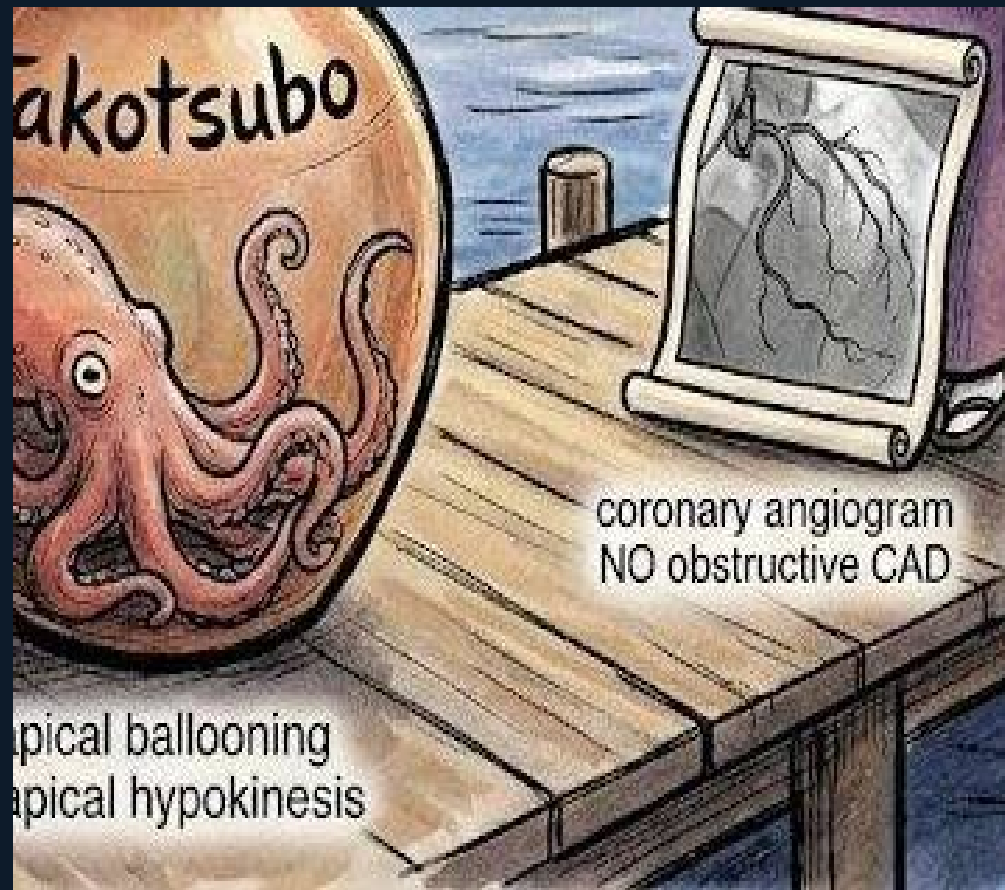
The octopus trap jar — wide at the bottom, narrow at the neck — is the LV in Takotsubo. The wide bottom: apical ballooning (hypokinetic apex). The narrow neck: basal hypercontractility. The stunned octopus inside with limp tentacles = apical hypokinesis. The pot shape is exactly what the LV looks like at end-systole on echo or ventriculography. The name Takotsubo is Japanese for 'octopus pot' — coined because the LV silhouette in this condition resembles this traditional fishing trap.



Scene F — Takotsubo | Japanese Fishing Village

Trigger: Catecholamine Surge

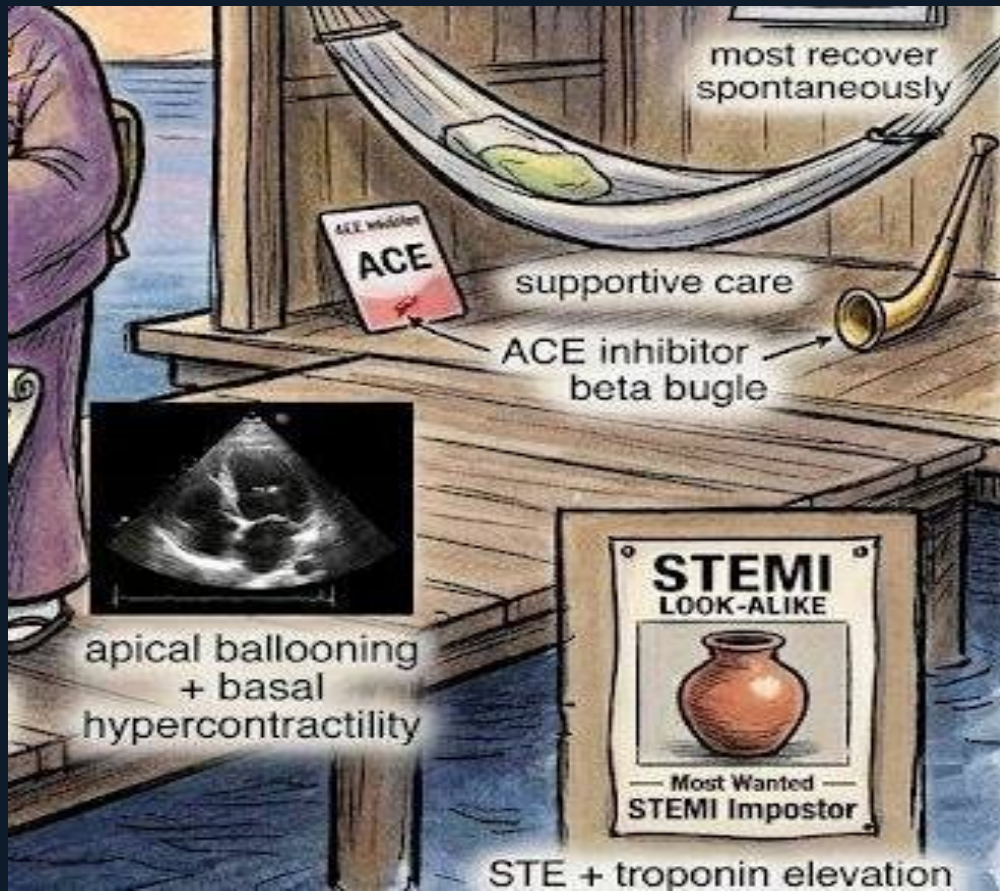
The postmenopausal woman receives terrible news — stress lightning bolt strikes her heart. Catecholamine storm cloud surrounds the myocardium. Takotsubo is triggered by acute emotional stressor (death of loved one, bad news, fear) or physical stressor (surgery, critical illness). Mechanism: massive catecholamine surge stuns myocardial cells, particularly the apex which has the highest density of sympathetic nerve terminals. Board demographic: postmenopausal women are disproportionately affected — estrogen's protective role in catecholamine signaling.



Scene F — Takotsubo | Japanese Fishing Village

Normal Coronaries + Apical Ballooning

The coronary angiogram scroll shows clear vessels — NO obstructive CAD. This is the diagnostic linchpin: the presentation mimics STEMI but catheterization shows normal coronaries. The echo shows apical ballooning with basal hypercontractility — distinguishes Takotsubo from anterior STEMI. In anterior STEMI, the LAD territory is infarcted and the apex is hypokinetic. In Takotsubo, the apex is hypokinetic despite normal coronaries — catecholamine stunning, not ischemia.



Scene F — Takotsubo | Japanese Fishing Village

Supportive Care + Recovery

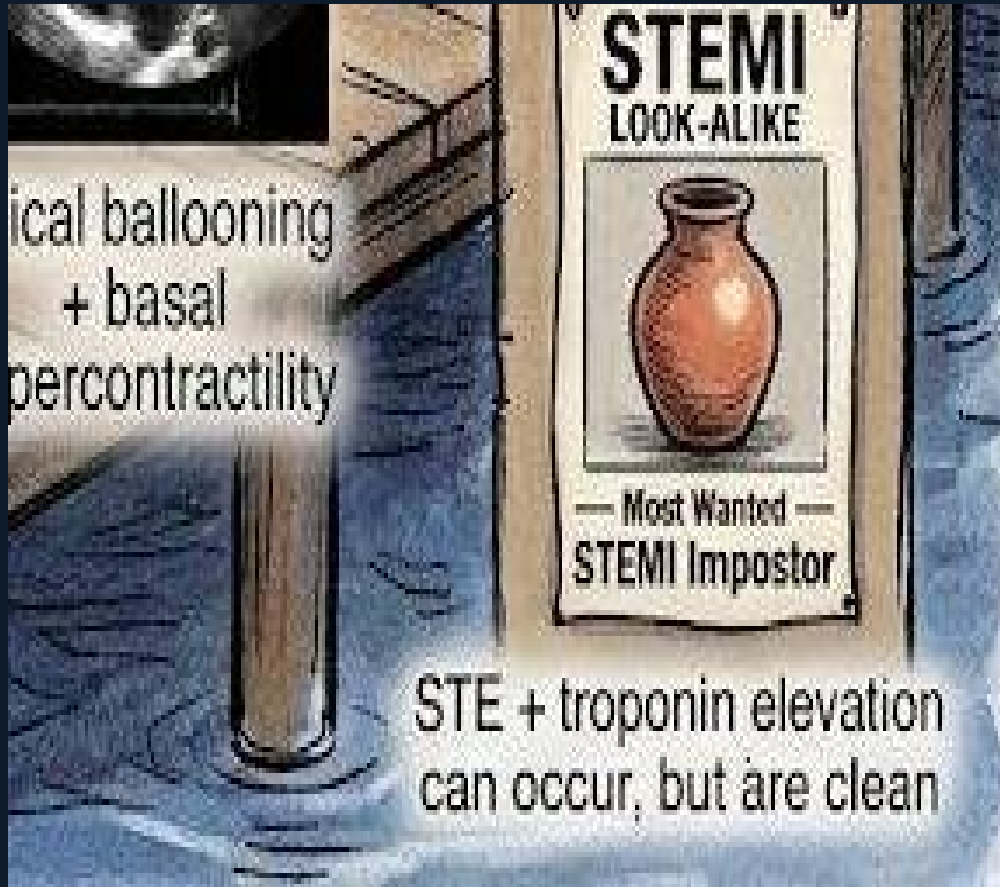
The treatment hut: supportive care, ACEi, beta-blocker for hemodynamic management. No specific Takotsubo therapy — treat complications as needed. Calendar: recovery in 1–4 weeks. Most patients recover to normal EF spontaneously. Board rule: 'What is the expected course of Takotsubo?' Spontaneous recovery in days to weeks — excellent prognosis. Board trap: 'Start aggressive HFrEF therapy long-term.' Wrong — EF normalizes within weeks; long-term GDMT is not indicated.



Scene F — Takotsubo | Japanese Fishing Village

Spiro Red X + QT Prolongation

Spironolactone red X: NOT primary treatment for Takotsubo — tested as a wrong-answer distractor on the boards. While MRAs are useful in HFrEF, they are not indicated as primary treatment for Takotsubo. QT prolongation rope: Takotsubo causes QT prolongation during the acute phase, creating torsades de pointes risk. Antiarrhythmic drugs that further prolong QT (Class IA, III) should be avoided acutely.



Scene F — Takotsubo | Japanese Fishing Village

STEMI Impostor: The Diagnostic Trap

The 'Most Wanted: STEMI Impostor' poster encodes the core scenario. ST elevation + troponin elevation CAN occur — but coronary arteries are clean. Board scenario: 68-year-old woman presents with acute chest pain and ST elevation after husband's death. Troponin is elevated. The correct management: emergent coronary angiography to exclude true STEMI — you cannot distinguish clinically. When coronaries are normal and echo shows apical ballooning → Takotsubo. Wrong answer: empiric thrombolytics without angiography.

Scene F — Quick Reference

Apical ballooning + basal hypercontractility

Wide-bottom octopus pot shape — characteristic

Postmenopausal women most affected

Estrogen loss → ↑ catecholamine sensitivity

Recovery in 1–4 weeks

Most recover spontaneously — excellent prognosis

QT prolongation risk

Avoid QT-prolonging antiarrhythmics acutely

Trigger: emotional or physical stress

Catecholamine surge stuns myocardium

Normal coronaries = diagnostic

Angio required to exclude true STEMI

Spironolactone: NOT primary treatment

Classic wrong-answer distractor

STEMI Impostor

STE + troponin + normal coronaries = Takotsubo